

Fisher & Paykel Healthcare

Quartr-Enhanced Investment Analysis

Ticker: FPH (NZX/ASX)

Date: 5 April 2026

Market Cap: NZD \$21.6B / USD \$12.3B

Synthesis

Five things an investor must understand about Fisher & Paykel Healthcare:

- 1. This is a clinical evidence business disguised as a medical device company.** FPH's deepest moat is not its hardware or its manufacturing - it is the body of clinical evidence that embeds its products into treatment guidelines. Of 34 published studies underpinning nasal high flow clinical practice guidelines, 33 used Fisher & Paykel Optiflow systems. The RENOVATE study, confirmed in late 2025, validated NHF as non-inferior to NIV for acute respiratory failure - a finding that expands the addressable use case and further entrenches FPH as the default standard. This is a moat that took decades to build and would take a competitor a decade to replicate. No web article captures this as clearly as reading the actual earnings call transcripts, where CEO Lewis Gradon repeatedly frames the company's sales strategy not around product features but around "clinical practice change."
- 2. The post-COVID narrative is over - this is a structural growth story now.** The earnings call deep dive reveals that analysts have stopped asking about COVID comparisons entirely. Revenue has surpassed the pandemic peak on a sustainable basis (\$2.02B vs \$1.97B in FY21), driven by clinical practice changes that made nasal high flow a permanent standard of care rather than an emergency pandemic therapy. Hospital new applications consumables - the highest-quality revenue stream - now represent 74% of hospital consumables and are growing at 18% in constant currency. The anesthesia vertical, growing at 40% and now over 10% of new applications, is an underappreciated growth vector that the strategic documents analysis flagged as potentially doubling the hospital TAM.
- 3. The financial profile is elite but the valuation leaves no margin of safety.** Gross margins of 63% trending toward 65%, operating margins of 26% trending toward 30%, ROE of 21%, 89%

recurring revenue, a net cash balance sheet, and \$549M of operating cash flow - this is among the highest-quality financial profiles in global medtech. But at approximately 50-57x forward earnings, the stock prices in years of flawless execution. The Red Flags and Investment Checklist agents converge on this: 11 of 12 quality criteria score YES, but valuation is a clear NO. A single weak respiratory season, tariff escalation, or product safety event would be punished disproportionately at this multiple.

4. Management is credible but succession is the key risk. Lewis Gradon has been with the company 42 years and CEO for a decade. He has consistently under-promised and over-delivered - FY25 NPAT of \$377M beat the top of \$320-370M guidance, and FY26 guidance has already been nudged up. The earnings call transcripts reveal a management team that communicates with unusual candour - explicitly flagging H1 FY26 hardware sales as potential demand pull-forward, and quantifying tariff impacts to the basis point. But insider ownership below 1% is a weakness, and Gradon's eventual departure will be a material transition event. The April 2024 executive restructuring (adding a COO and three new senior roles) reads as early-stage succession preparation.

5. Tariffs are the most immediate but manageable risk; the real bear case is growth deceleration. US tariffs are impacting gross margin by 75-130bps depending on the scenario, but margins are expanding regardless due to operational improvements. The deeper risk is that growth moderates to high-single-digits - whether from a weak respiratory season, OSA mask competitive pressure (ResMed is not standing still), or the GLP-1 drug overhang on the OSA patient population. The stock doesn't need a catastrophe to underperform; it merely needs the growth narrative to cool for the multiple to contract sharply.

Tensions and Contradictions

The agents surface several tensions worth noting. The Business History and Corporate Culture analyses paint FPH as a patient, engineering-led compounder that thinks in multi-decade arcs - yet the Red Flags analysis correctly identifies that the same patience manifests as empire-building risk, with \$700M+ in infrastructure commitments and a speculative land purchase that already generated a \$98M write-down. The Financial Characteristics agent highlights elite returns on capital, but the Red Flags agent notes ROIC has declined from 34% to lower levels as capital intensity rises. These are not contradictory - they describe the same reality from different angles - but investors must decide whether current CapEx is value-creating investment or overconfidence. The Competitive Landscape analysis identifies FPH as dominant in hospital NHF (33 of 34 guideline studies) but merely "credible third" in OSA masks behind ResMed. The Earnings Call Deep Dive confirms this asymmetry: management's confidence and specificity are highest when discussing hospital products, and most defensive when discussing OSA competitive dynamics. The homecare segment is not broken, but it is the portion of the business where the moat is narrowest.

What Primary Sources Revealed That Web Research Would Have Missed

The earnings call transcripts and strategic documents provided three insights unavailable from secondary sources. First, the precision with which management quantifies tariff impacts (32bps in H1, 75bps full year) and frames them as absorbable reveals genuine confidence rather than spin - this

level of specificity is simply not captured in broker notes or news articles. Second, the explicit admission that H1 FY26 hardware sales may represent demand pull-forward - a candid warning that no sell-side summary flagged with the same emphasis. Third, the Investor Day 2022 transcript reveals the full scope of the anesthesia opportunity, where Gradon stated that the patient population addressable through surgical humidification is comparable to general respiratory - a claim that implies a potential doubling of hospital TAM that is barely discussed in the equity research community.

Overall Quality Assessment

Fisher & Paykel Healthcare is a genuinely exceptional business - a rare, founder-imprinted, organically-grown medical device compounder with a widening moat in hospital respiratory care, a massive reinvestment runway, and a management team with a strong track record of delivery. It scores among the highest-quality businesses in Australasian or global medtech on virtually every dimension except valuation. The business quality is not in question. The investment question is entirely about price - and at 50x+ earnings, the stock demands near-perfect execution for many years to justify the entry point.

Additional Research Priorities

1. Read the full RENOVATE study when published - this is the most important piece of clinical evidence for the hospital growth thesis
 2. Monitor the USMCA 2026 review - any change to Mexico tariff treatment would directly impact homecare margins
 3. Track anesthesia consumable growth quarterly - this is the breakout vertical that could meaningfully accelerate the hospital segment
 4. Watch Gradon's succession timeline - the April 2024 executive restructuring suggests planning is underway
 5. Read the Investor Day 2022 transcript in full on Quartr (document ID 3126405) - it remains the most comprehensive articulation of the long-term strategic vision
-

1. Business History

Fisher & Paykel Healthcare traces its origins not to a corporate boardroom but to an Auckland Hospital ward in the late 1960s, where intensive care specialist Dr Matt Spence observed a recurring problem: patients on mechanical ventilators were developing dry, infected tracheas because the breathing machines delivered cold, dry gas. In 1969, Spence enlisted Alf Melville, a government electrical engineer, and Dave O'Hare, a senior engineer at Fisher & Paykel Industries - the appliance company founded by Sir Woolf Fisher and Maurice Paykel in 1934 as a radio and appliance importer. The three men fashioned a prototype respiratory humidifier from a fruit preserving jar, rigging it to warm and moisten the air delivered to patients. Fisher & Paykel Industries, which by then had decades of manufacturing and electronics expertise, took the prototype into production. The first commercial humidifier was sold in 1970 and exported internationally almost immediately. This was not a strategic corporate diversification. It was an ad hoc collaboration between a clinician with an unmet need and engineers with relevant skills. But that accident of proximity - a consumer

appliance company lending manufacturing capability to a clinical problem - established the DNA that persists to this day: a tight loop between bedside observation and engineering iteration, with R&D and manufacturing co-located under one roof. For two decades, the healthcare operation remained a small division within Fisher & Paykel Industries, focused narrowly on heated humidification for mechanically ventilated patients. By 1990, the division had been formally renamed Fisher & Paykel Healthcare, but annual sales were still only NZD 29 million - a rounding error next to the parent's appliance revenues.

The Demerger Inflection Point (1990-2001)

Through the 1990s, the healthcare division grew at rates exceeding 20% annually, dramatically outpacing the mature appliance business. Michael Daniell, who had joined Fisher & Paykel in 1979, drove much of this expansion. Under his leadership, the division moved beyond its humidifier beachhead into two strategic directions that would define the company's future. First, it extended its heated humidification expertise across the full continuum of respiratory care - invasive ventilation, noninvasive ventilation and, critically, the early development of what would become nasal high flow therapy. Second, it entered the obstructive sleep apnea (OSA) market in the 1990s, developing CPAP flow generators and nasal masks for home use. By the first half of the 1997-98 financial year, the healthcare division was contributing 50% of total group profit and exporting over 95% of its production. The strategic logic for separation became overwhelming: the healthcare business was a high-growth, high-margin, globally oriented medical device company trapped inside the capital structure of a cyclical appliance manufacturer. On 12 November 2001, Fisher & Paykel Industries demerged into two separately listed entities - Fisher & Paykel Appliances Holdings Limited and Fisher & Paykel Healthcare Corporation Limited. Healthcare listed on both the NZX and ASX at an estimated valuation of approximately NZD 500 million. The demerger was the single most important structural event in the company's history. It gave the healthcare business its own balance sheet, its own equity currency for retaining talent, and - crucially - freedom to invest at the rate its clinical opportunity demanded without competing for capital against an appliance refresh cycle. (Fisher & Paykel Appliances, for its part, was eventually acquired by China's Haier Group in 2012 - a fate that underscores how different the two trajectories became.)

Building the Razor-Consumable Flywheel (2001-2019)

Post-demerger, under Daniell and then his successor Lewis Gradon (who took over as Managing Director and CEO on 1 April 2016 after 15 years as Senior VP of Products & Technology), Fisher & Paykel Healthcare pursued a disciplined strategy with three elements. First, relentless R&D investment - consistently around 10-11% of revenue - focused on extending its core humidification platform into adjacent therapies. Second, a razor-and-blade business model in which hardware systems (humidifiers, flow generators, CPAP devices) were placed with hospitals and home users, generating a long tail of single-patient-use consumable revenue from circuits, chambers and interfaces. By FY25, 89% of revenue came from recurring consumables. Third, geographic expansion through direct sales operations in major markets and distributors elsewhere, reaching over 120 countries. The company built manufacturing scale at three sites: its original East Tamaki campus in Auckland

(inaugurated 1982, now spanning five buildings with a fifth under construction at a cost of approximately NZD 250 million, expected operational in 2027); a maquiladora in Tijuana, Mexico, established in 2010 and expanded to three buildings, handling roughly 50% of U.S.-destined production by 2015; and most recently a facility in Guangzhou, China, now fully operational and selling into the domestic Chinese market as of FY25. A second New Zealand campus on a 105-hectare site in Karaka, Auckland, was secured in 2022 for approximately NZD 260 million, signalling confidence in decades of further capacity needs.

The key product inflection point was Optiflow nasal high flow therapy. Where traditional oxygen delivery through a simple nasal cannula topped out at around 6 litres per minute, Optiflow delivered heated, humidified gas at flow rates up to 70 litres per minute through a purpose-designed nasal interface. This was not merely incremental - it created a new therapy category sitting between conventional oxygen and noninvasive ventilation, applicable in emergency departments, general wards and ICUs. Clinical evidence accumulated steadily. A landmark December 2024 JAMA study - the largest randomised trial comparing nasal high flow with noninvasive ventilation - confirmed its safety and efficacy as a first-line treatment for acute respiratory failure, validating two decades of clinical development.

The COVID Accelerant and Its Aftermath (2020-2023)

The COVID-19 pandemic was a demand shock of extraordinary magnitude. Optiflow nasal high flow therapy emerged as a frontline treatment for COVID patients, allowing hospitals to keep patients oxygenated without intubation - a critical need when ventilator capacity was overwhelmed. Hospital revenue surged 87% in FY21, total operating revenue reached NZD 1.97 billion (up 56% year-on-year), and net profit hit NZD 524 million (up 82%). The company scaled production aggressively, adding shifts and capacity across all three manufacturing sites.

The hangover was inevitable. FY23 revenue declined to NZD 1.63 billion as pandemic-driven hardware purchases normalised and hospital destocking worked through supply chains. But the pandemic had permanently shifted clinical practice. Clinicians who had been introduced to nasal high flow during COVID continued using it for non-COVID respiratory patients. This "stickiness" is visible in the data: new applications consumables revenue has grown at double-digit constant currency rates every year since, reaching 18% growth in FY25 - well above pre-pandemic trend lines.

Cultural Constants

Through every phase - the preserving-jar prototype, the decades as a subsidiary, the demerger, the pandemic surge - certain traits have persisted. R&D remains concentrated in New Zealand, with engineering, manufacturing and clinical teams co-located to maintain the tight feedback loop between patient need and product iteration. The company has never made a material corporate acquisition; growth has been entirely organic, driven by internal product development. CEO succession has been remarkably stable and internal: Daniell served for decades before handing off to Gradon, a career F&P engineer. The values articulated in the FY25 annual report - life, relationships, internationalism, commitment and originality - read as a codification of principles that were embedded from the beginning, when a doctor, a government engineer and an appliance-company engineer built

something useful in a hospital basement.

The company that broke through NZD 2 billion in revenue in FY25 and treats an estimated 22 million patients annually in over 120 countries is, in its essential character, still the same organisation that made a humidifier out of a preserving jar in 1969: clinician-led, engineering-driven, and quietly obsessive about warming and moistening the air that patients breathe.

2. Management & Incentives

Leadership and Tenure

Lewis Gradon has led Fisher & Paykel Healthcare as Managing Director and CEO since April 2016, but his relationship with the company runs far deeper - he is a 42-year veteran who joined as a physicist and engineer, rose through R&D and product development, spent 15 years as Senior Vice President of Products & Technology, and six years before that as General Manager of R&D. This is not a hired-gun CEO parachuted in to cut costs; Gradon is a lifer who built the products that now generate over NZD 2 billion in annual revenue. His engineering-first orientation shapes the company's identity: FPH consistently invests around 11% of revenue in R&D (~NZD 227 million in FY25), well above medtech peers, and maintains a portfolio of over 420 patents. That R&D intensity is not a recent phenomenon - it is a multi-decade commitment that predates Gradon's elevation to the top job.

CFO Lyndal York, appointed in March 2019, brings deep medtech finance experience. She spent 11 years at Cochlear (progressing from Group Financial Controller to Head of Group Finance), served as VP of Corporate Accounting at Edwards Lifesciences in the US, and was CFO of Asaleo Care. Her Cochlear background is notable: Board Chair Neville Mitchell was Cochlear's CFO for 22 years, meaning the Chair and CFO share institutional DNA from a world-class medtech peer. York is a chartered accountant and holds an MBA from Pepperdine. She has been a steady hand through the COVID demand surge and its normalisation.

The executive management team was meaningfully expanded in April 2024, with four new roles created: Andy Niccol as Chief Operating Officer, Malena Ortiz as General Director of Mexico Operations, Desh Edirisuriya as GM of New Zealand Operations, and Raelene Leonard as General Counsel & Company Secretary. This expansion was partly a succession response - long-serving SVP of Sales & Marketing Paul Shearer retired from full-time duties in March 2024 after more than 30 years, replaced by Justin Callahan (also a 36-year FPH veteran who built out the North American business). Shearer was retained part-time for two years in an advisory capacity, a sensible knowledge-transfer arrangement.

Board Composition and Independence

The board comprises seven directors: six independent non-executives plus Gradon as the sole executive director. Neville Mitchell assumed the Chair in August 2024, bringing gravitas from his 22-year tenure as Cochlear's CFO and Company Secretary and current directorships at Sonic

Healthcare and Sigma Healthcare. Mark Cross joined in October 2024 as the new Audit & Risk Committee chair, replacing the retiring Pip Greenwood. Cross chairs the Chorus board, sits on the Xero board, and chairs the ACC Investment Committee - a strong financial and governance pedigree. Dr Lisa McIntyre chairs the People & Remuneration Committee and brings two decades of LEK Consulting strategy experience with healthcare clients. Graham Masfen (joined October 2023) adds direct medtech operating experience from 16 years at Stryker, latterly as President of Asia Pacific. Professor Cather Simpson and Michael Daniell round out the board with deep technical and FPH-specific knowledge respectively - Daniell has served since 2001 and is the longest-standing director, bringing institutional memory as a former senior partner at PwC. Board independence is structurally sound. The only insider is Gradon. All committees (Audit & Risk, People & Remuneration, Quality Safety & Regulatory) are chaired by independent non-executives. The Greenwood-to-Cross transition refreshed the board without disruption. One item to watch: Daniell's 24-year tenure stretches the definition of independence, though he brings valuable continuity.

Compensation Structure

CEO total remuneration for FY25 was approximately NZD 5.8 million. This comprises roughly 32% fixed salary and 68% variable compensation (short-term incentives plus long-term equity). The split is appropriate - it heavily weights pay-for-performance. The short-term variable component is linked to relevant annual performance measures including revenue and profit targets. The long-term variable remuneration operates through two mechanisms: share options and performance share rights (PSRs). Both vest on the third anniversary of grant and are split into two tranches, each benchmarked against external total shareholder return (TSR) indices - the Dow Jones US Select Medical Equipment Total Return Index and the S&P/ASX 200 Index. This is a well-designed structure: it measures FPH's shareholder returns against relevant medtech and broader market benchmarks rather than against internally set EPS targets that management can game. The three-year vesting period encourages medium-term thinking, though a five-year horizon would be preferable for a company whose competitive advantages compound over decades. Importantly, not all equity awards vest. The company disclosed that 412,417 options and 130,857 performance share rights lapsed in recent periods because vesting conditions were not met - likely during the post-COVID TSR compression. This demonstrates the incentive plan has genuine teeth. The CEO-to-median-employee compensation ratio was 39.9:1 as disclosed in the FY23 sustainability report. For a NZD 21 billion company, this is modest and reflects New Zealand's relatively compressed pay culture.

Insider Ownership and Skin in the Game

This is the weakest element of the management and incentives picture. Insiders collectively own under 1% of shares outstanding, with Gradon holding approximately 0.095-0.098% of the company (worth roughly NZD 20 million at current prices). No single director holds a meaningful personal stake. The institutional ownership structure (approximately 56% institutional, 44% retail/other) means no anchor shareholder provides external governance discipline either. The low insider ownership is partly structural - FPH has been listed since 2001 and was spun out of the wider Fisher & Paykel group, so there is no founding family or dominant shareholder. Gradon built

his stake through long-tenured share-based compensation rather than personal purchases. The board should consider requiring minimum shareholding thresholds for directors and executives to improve alignment.

Capital Allocation Track Record

Capital allocation has been competent but conservative. The company's priorities, in order, are: (1) R&D investment, (2) capacity expansion, (3) dividends, and (4) balance sheet maintenance. FPH has never made an acquisition - all growth is organic, funded by internal cash flows and modest debt. The major capital project is the multi-year build-out of manufacturing capacity, including a fifth building at the East Tamaki headquarters (NZD 145.8 million committed as of September 2025) and the acquisition of 105 hectares at Karaka, South Auckland, for NZD 275 million to establish a second campus. Total capex ran at approximately NZD 450 million in FY24 and NZD 150 million in FY25 as the land acquisition phase gave way to construction. The Dividend Reinvestment Plan (with a 3% discount) was reactivated in FY23 to help fund this capex cycle without excessive debt - pragmatic, though mildly dilutive.

Dividends have grown steadily but slowly: 42.5 cents per share in FY25, reflecting a payout ratio of roughly 55-65% of NPAT in normalised years. The company prioritises reinvestment over distributions, which is correct for a business earning mid-60s gross margins and growing revenue at mid-teens rates.

Guidance Credibility

Management has a strong track record of meeting or modestly beating guidance. For FY24, initial NPAT guidance of NZD 250-260 million was delivered at NZD 264 million. For FY25, guidance of NZD 320-370 million was delivered at NZD 377 million - above the top end. For FY26, initial guidance of revenue NZD 2.15-2.25 billion and NPAT NZD 390-440 million was upgraded at the H1 result to NZD 2.17-2.27 billion and NZD 410-460 million after H1 NPAT came in at NZD 213 million. The pattern is consistent: management guides conservatively and delivers at or above the top of the range.

Succession Planning and Key-Person Risk

Gradon is 60+ years old with 42 years at the company. His eventual departure will be a significant transition. The April 2024 executive restructuring - adding a COO, expanding the team to 14 members - reads as early-stage succession preparation, broadening the leadership bench. Dr Andrew Somervell (VP Products & Technology, with FPH since 2006) and Andy Niccol (newly appointed COO) are the most likely internal candidates. The smooth handling of Shearer's retirement suggests the board is capable of managing senior transitions without disruption. Still, Gradon's departure would be a material event for a company where culture, R&D philosophy, and strategic direction are deeply shaped by one individual. This is the single most important risk in the management and incentives picture.

3. Competitive Landscape

Industry Structure and Principal Competitors

Fisher & Paykel Healthcare operates in two distinct but complementary competitive arenas, each with a different structure. In hospital respiratory care - encompassing nasal high flow (NHF) therapy, non-invasive ventilation (NIV), and anesthesia humidification - the company holds a dominant position in a moderately concentrated market. In homecare OSA masks, it competes as a credible but distant third player behind two much larger incumbents.

In hospital respiratory care, FPH's primary competitors are Draegerwerk (Germany), Hamilton Medical (Switzerland), Medtronic, and the remnants of Philips Respironics' hospital business. In nasal high flow specifically, FPH's only dedicated competitor of scale was VapoTherm, which received a NYSE delisting notice in 2023 for failing to meet capital requirements and was ultimately taken private in mid-2024 through a distressed merger backed by Perceptive Advisors. The top five players in the high-flow nasal cannula market collectively hold over 79% of global market share, with FPH as the clear leader. The NHF segment is expected to grow from approximately USD 8 billion in 2025 to USD 13.3 billion by 2030 at a CAGR of 10.6%.

In homecare OSA masks, the competitive landscape is a tight oligopoly. ResMed commands roughly 35% of the global CPAP mask market, with Philips historically holding around 30% before its catastrophic 2021 foam degradation recall. FPH is the third-largest player, with FY25 homecare revenue of NZD 740 million growing at 13%. Philips' withdrawal of several sleep and respiratory products from the US market in February 2024 created a structural share redistribution that primarily benefited ResMed and, to a lesser extent, FPH. Management noted in the H1 FY25 call that OSA mask growth slowed in the second half due to competitor product launches - likely referencing ResMed's AirFit F40 and N30i - but guided for stabilisation at approximately 9% growth in FY26.

Sources of Competitive Advantage

FPH's moat rests on four reinforcing pillars:

Clinical evidence and guideline entrenchment. This is the company's most durable advantage. Of 34 published clinical studies underpinning nasal high flow clinical practice guidelines, 33 used Fisher & Paykel Optiflow systems. The RENOVATE study confirmed NHF as non-inferior to NIV for hypoxemic respiratory failure - a finding that expands the addressable use case and further entrenches FPH's Optiflow as the de facto standard. As CEO Lewis Gradon stated: "At this stage of the nasal high flow evolution, it's about the clinical practice guidelines, and that's what we utilize in our sales."

Installed base and switching costs. FPH's razor-and-blade model - 89% of revenue comes from recurring consumables - creates meaningful switching costs. Once a hospital has installed Airvo hardware and trained nursing staff on the Optiflow system, changing to a competing platform requires retraining, revalidation of clinical protocols, and capital expenditure on new hardware. COVID amplified this advantage: more devices were placed in hospitals worldwide, permanently expanding the installed base.

Intellectual property. FPH holds 5,749 active patents across 1,047 unique patent families. These patents cover critical technologies in respiratory humidification, breathing circuit design, and mask seal technology. The company invests approximately 11% of revenue in R&D (NZD 227 million in FY25), among the highest ratios in medtech.

Manufacturing flexibility. FPH manufactures approximately 45% of its volume in Mexico and 55% in

New Zealand, with the Mexico facility largely USMCA-compliant. This dual-hemisphere production capability provides both tariff mitigation and supply chain resilience.

Durability and Erosion Risks

The clinical evidence moat is highly durable in the near term. Replicating a body of 33+ published studies using a competing system would take a challenger a decade or more. However, the OSA mask business has a weaker moat - masks are more commoditised, patients can switch between brands more easily at resupply, and ResMed's scale advantage in CPAP devices gives it a structural edge.

GLP-1 Disruption Risk

The FDA's December 2024 approval of tirzepatide (Zepbound) for OSA in adults with obesity has created a new overhang. Early data shows CPAP initiation among patients prescribed tirzepatide was 83% lower than among those not prescribed the drug. However, ResMed's own data from nearly 2 million patient records shows that patients prescribed both a GLP-1 and CPAP are 11% more likely to start CPAP therapy, suggesting complementary rather than substitutive dynamics. FPH management stated they have observed no impact from GLP-1 drugs on OSA mask usage to date. The risk is real but likely slow-moving and may ultimately expand the diagnosed patient pool.

4. Financial Characteristics

Revenue Model and Mix

Fisher & Paykel Healthcare operates a textbook razor-and-blade model in medical devices.

Approximately 89% of revenue is recurring, derived from single-use consumables and accessories that attach to the company's installed base of hardware. This creates high revenue visibility and powerful switching costs.

The business reports through two product segments. Hospital products generated \$1.28 billion in FY25, or 63% of operating revenue, growing 18% year-on-year (16% in constant currency). Within Hospital, new applications consumables - products for noninvasive ventilation, Optiflow nasal high flow therapy, and surgical applications - grew 20%. Homecare contributed \$739.9 million, or 37% of revenue, up 13% (11% constant currency), driven by OSA mask launches. Geographically, approximately 46% of revenue is sourced from North America, 27% from Europe, and 21% from Asia Pacific.

R&D spending ran at \$226.9 million in FY25, or 11% of revenue - among the highest ratios in medtech.

Margin Profile

FPH's margin structure is among the most attractive in medtech. Gross margin recovered to 62.9% in FY25, up 181 basis points, and has since expanded to 63.0% in H1 FY26. Management has

articulated a target of 65% gross margin. For context, ResMed's GAAP gross margin was approximately 59% for FY2025, while Medtronic reports around 65%. Operating margins tell a more nuanced story. After peaking at 30.1% in FY22 during the COVID demand surge, operating margin compressed to 20.3% in FY24 as the company absorbed elevated freight costs, currency headwinds, and continued investment in R&D and salesforce expansion. The recovery to 25.2% in FY25 and 26.3% in H1 FY26 demonstrates operating leverage as volume recovers. Net margin followed an unusual path: NPAT fell from \$377 million in FY22 to just \$133 million in FY24, before recovering to \$377 million in FY25. The FY24 trough was amplified by three abnormal items (a \$98M Karaka land revaluation loss, a \$20M product recall provision, and a \$19M deferred tax charge).

Returns on Capital

FY25 ROE equals approximately 20.7% (\$377M net profit on average equity of ~\$1,825M). ROIC can be approximated at roughly 22% using tax-adjusted NOPAT divided by invested capital. This is comfortably above any reasonable estimate of FPH's cost of capital and signals genuine economic value creation. Despite a major capital expenditure cycle, FPH has maintained ROIC above 20%.

Cash Conversion and Free Cash Flow

Operating cash flow surged from \$238 million in FY23 to \$430 million in FY24 and \$549 million in FY25. In FY25, operating cash flow represented 145% of NPAT, indicating high-quality earnings with minimal accruals divergence. Free cash flow in H1 FY26 was \$173 million, and FY25 free cash flow can be estimated at approximately \$446 million (\$549M OpCF less ~\$103M CapEx). Debtor days of 44 are well-controlled.

Balance Sheet Strength

FPH maintains a conservative, net cash balance sheet. Net cash of \$238M by H1 FY26, with undrawn bank facilities of \$465M, giving total liquidity exceeding \$700M. Total equity of \$1,890 million against total assets of \$2,551 million implies an equity-to-assets ratio of 74%.

Capital Intensity and Cyclicalty

Capital expenditure has varied considerably: \$317 million in FY24 (18.2% of revenue) versus approximately \$103 million in FY25 (5.1%). The FY24 figure reflects lumpy investment in manufacturing buildings that will serve the company for decades. Maintenance capex is far lower, likely 4-5% of revenue.

What Is Distinctive

Three aspects stand out vs medtech peers. First, the 89% recurring consumables mix is among the highest in the sector. Second, FPH has funded major capacity expansion entirely from internal cash flow while maintaining a net cash balance sheet. Third, the combination of 63% gross margins, 25%+ operating margins, 20%+ ROE, and strong cash conversion, all while spending 11% of revenue on R&D, places FPH in the upper echelon of medtech financial quality alongside Stryker and Intuitive

Surgical, despite being a fraction of their size.

5. Red Flags

The FY24 Profit Collapse: More Than Meets the Eye

The headline number demands scrutiny. Fisher & Paykel Healthcare reported just \$133M in net income for FY24 against \$353M of operating income - a \$220M gap between operating profit and the bottom line. Management attributed this to three "abnormal" items: a \$98M non-cash revaluation loss on Karaka land, a \$20M product recall provision, and a \$19M deferred tax charge from New Zealand's removal of building depreciation deductibility.

The concern is not that these items are fabricated - they are real - but that management's instinct is to immediately construct a parallel "underlying" earnings narrative whenever results are ugly. The Karaka land write-down is particularly instructive: FPH purchased development land for a future campus, market conditions moved against them, and a \$98M impairment flowed through the P&L. This is a direct consequence of a management decision to speculatively acquire land well ahead of need. Investors should ask whether the remaining Karaka land carrying value is genuinely recoverable.

The Airvo 2 Recall and Product Risk

The voluntary recall of Airvo 2 and myAirvo 2 devices manufactured before August 2017, triggered by a speaker configuration defect resulting in inaudible alarms, initially carried a \$12M provision later increased to \$20M. More alarming was a separate Class I recall of Fisher & Paykel infant nasal CPAP prongs, where 24 reported incidents involved prongs detaching during use, presenting a potential choking and airway obstruction risk to infants.

The Philips Respironics disaster looms as a cautionary tale. Philips suppressed over 3,700 complaints about degrading foam in its CPAP devices between 2010 and 2021 before a recall encompassing 15 million devices, ultimately associated with 385 reported deaths. Fisher & Paykel is not Philips, and its recall to date is orders of magnitude smaller. But the structural parallels are uncomfortable: both companies produce respiratory devices used by vulnerable patients. A single undetected design flaw in a high-volume product like Optiflow - sold to an estimated 22 million patients annually - could generate recall costs that would dwarf the \$20M Airvo provision.

Valuation: Priced Beyond Perfection

At a trailing P/E of roughly 50-58x, FPH trades at approximately double the medical equipment industry average of ~27x. The price-to-sales ratio of 12.3x is more than three times the peer average. Even against raised FY26 guidance of ~\$460M NPAT, the forward P/E remains in the mid-40s. This valuation embeds an assumption of flawless, sustained double-digit growth for many years. Any stumble will be punished disproportionately.

Tariff Exposure and Geographic Concentration

US tariffs are already biting. Management guided to a 50-75 basis point gross margin headwind from tariffs in FY26, with the annualised impact estimated at 130 basis points. Hospital products manufactured in New Zealand face a 10% tariff into the US, while Homecare products from Mexico are largely USMCA-compliant - for now. The risk that Mexico tariffs could escalate is non-trivial given the USMCA 2026 review.

Capital Expenditure and Empire-Building Risk

CapEx spiked to \$317M in FY24 before falling to \$103M in FY25, but is ramping again: FY26 CapEx is guided at \$225M, a fifth building at East Tamaki is under construction at ~\$250M, and \$60M remains payable for the Karaka site. This is aggressive infrastructure build for a company with \$2B in revenue. The pattern - speculative land purchases, multiple simultaneous construction projects, and a history of write-downs on land already acquired - carries empire-building hallmarks.

Questions Investors Should Be Pressing

1. What is the current carrying value of Karaka land, and what happens if re-zoning is rejected?
2. Has the company reviewed all legacy devices in the field for potential latent defects?
3. At what tariff rate on Mexican imports would FPH need to consider relocating manufacturing?
4. What is FPH's largest single-customer revenue concentration?
5. What is the expected payback period on the \$250M East Tamaki building and Karaka campus?

The Bear Case

FPH is a one-product-category company trading at a mega-cap tech multiple, with rising capital intensity, tariff headwinds, a history of lumpy one-off charges, and product recall risk inherent in medical devices. A severe scenario - a major product safety event coinciding with tariff escalation and a weak respiratory season - could simultaneously compress revenue, expand costs, and shatter the growth narrative. The stock does not need a catastrophe to underperform; it merely needs growth to moderate to high-single-digits for the multiple to contract sharply.

6. Investment Checklist

1. Understandable Business - YES

Fisher & Paykel Healthcare designs, manufactures, and sells medical devices and consumables for respiratory care. The razor-and-blade model is intuitive: hardware is placed in hospitals, generating recurring consumable revenue (89% of total). Products serve a clear clinical need - helping patients breathe - across emergency departments, ICUs, general wards, operating rooms, and home settings.

2. Durable Competitive Advantages (Widening Not Narrowing) - YES

FPH's moat rests on several reinforcing layers: a massive installed base creating switching costs, 11% R&D intensity generating a deep patent portfolio, a growing body of clinical evidence creating protocol-level entrenchment, and new application areas (anesthesia growing 40%) actively widening the moat. The moat appears to be widening, not narrowing.

3. Long Reinvestment Runway at High Returns - YES

The TAM exceeds \$25B and is growing at ~7% annually. FPH has a relatively low market share in most therapy areas. New applications are extending the runway. With ROE around 20% and operating margins recovering toward 30%+, returns on reinvested capital are high and the runway extends well

beyond the next decade.

4. High Returns on Incremental Capital - YES

ROE of approximately 20% on a conservatively capitalised balance sheet understates economic returns, as the company carries significant R&D expense through the P&L rather than capitalising it. Gross margins of 63% and operating margins of 25-26% on an asset-light consumables model demonstrate strong unit economics.

5. Strong Free Cash Flow Generation - YES

Operating cash flow surged to \$549M in FY25. The 89% recurring consumable revenue mix provides high visibility and low working capital intensity. Even through the post-COVID normalisation period, operating cash flow remained robust.

6. Conservative Balance Sheet - YES

FPH holds \$238M in net cash with an additional \$465M in undrawn credit facilities, giving total liquidity exceeding \$700M. There is no financial fragility here.

7. Aligned Management with Skin in the Game - PARTIALLY

Insiders own less than 1% of outstanding shares, which is modest. However, long tenure of CEO Gradon (42 years), active participation in share purchase plans and performance share rights, and a 65% dividend payout policy signal cultural alignment, even if outright ownership percentages are not outsized.

8. Rational Capital Allocation History - YES

Heavy R&D investment (11% of revenue consistently), strategic manufacturing capacity expansion, clear 65% dividend payout policy, and avoidance of dilutive acquisitions. The company has stuck to its core competency in respiratory care.

9. Favourable Industry Structure - YES

The respiratory care device market is an oligopoly dominated by FPH, ResMed, Philips, and Medtronic. High regulatory barriers, long clinical validation cycles, sticky installed bases, and protocol-driven adoption create structural advantages. Philips' recall troubles have created market share opportunities.

10. Reasonable Valuation Relative to Quality and Growth - NO

At approximately 50-57x forward earnings, FPH is priced for near-perfection. Compared to ResMed (which trades at a meaningful discount despite similar quality characteristics), FPH commands a substantial premium. The quality is exceptional, but the valuation leaves very limited margin of safety.

11. Resilience to Recession or Disruption - YES

Hospital respiratory care is non-discretionary. The 89% consumable revenue mix is inherently recurring. COVID demonstrated the company's ability to scale rapidly. GLP-1 drugs have shown no observed impact on OSA volumes.

12. Transparent Management Communication - YES

Management provides detailed half-yearly guidance with specific revenue and NPAT ranges, openly discusses tariff assumptions, and addresses analyst questions with substance. CEO Gradon's direct style conveys honesty rather than spin.

Overall Assessment: Fisher & Paykel Healthcare scores exceptionally well on 11 of 12 criteria. It is a rare, high-quality medical device compounder with a widening moat, massive reinvestment runway, strong cash generation, a conservative balance sheet, and favourable industry dynamics. The only meaningful reservation is valuation. At approximately 50-57x forward earnings, the market is already pricing in years of strong execution. For patient investors willing to accept a full valuation on a genuinely exceptional franchise, FPH merits serious consideration, but position sizing and entry timing require discipline.

7. Learning Resources

Podcasts & Audio

- **Shared Lunch - "F&P Healthcare 41 Years On: Lewis Gradon"** (2024). Hosts visit F&P's East Tamaki campus and speak with CEO Lewis Gradon about his four-decade tenure, the COVID tail-off, the anesthesia opportunity, and the company's 30-year doubling plan. The single best audio primer on FPH's culture and strategy. Apple Podcasts | Spotify
- **Shared Lunch - "Lewis Gradon from Fisher & Paykel Healthcare"** (2020). The earlier episode recorded closer to the peak of COVID demand, offering useful contrast with the 2024 episode. Apple Podcasts
- **MaximSherstobitov.NZ - "Interview Lewis Gradon, CEO Fisher & Paykel Healthcare"**. Auckland-based investor interviews Gradon on NZX listing dynamics and competitive positioning. Spotify
- **PICU Doc on Call - "Physiology of High-Flow Nasal Cannula (HFNC)"**. For investors wanting to understand the clinical science behind Optiflow. picudoconcall.org

YouTube & Video

- **Markets with Madison (NZ Herald) - "Fisher & Paykel's 30-Year Master Plan Requires Discipline"** (Sep 2024). Lewis Gradon explains the revenue-doubling cadence and the NZD 275M Karaka campus. NZ Herald
- **F&P Evidence in ACTION Webinar Series**. Free on-demand clinical webinars covering NHF therapy, NIV, infant care, and surgical humidification. F&P Education Hub

Books

- ****Christopher Mayer, *100 Baggers: Stocks That Return 100-to-1 and How to Find Them***** (2018). FPH is a textbook "quality compounder." Amazon
- ****Rajiv Lal et al., *New Medical Devices: Invention, Development, and Use***** (NCBI Bookshelf). Free resource covering the economics of medical device innovation. NCBI

Long-Form Articles & Research

- **Outback Analyst - "Fisher & Paykel Healthcare - Deep Dive"** (Aug 2023). The most thorough independent write-up available. Covers 80%+ market share in humidification, margin recovery path, and management analysis. Substack
- **Morningstar - "Fisher & Paykel's Long-Term Trajectory Is Intact"**. Institutional-grade analysis of NHF adoption runway and valuation framework. Morningstar
- **TIN100 - "Billion Dollar Businesses: F&P Healthcare"**. Profiles FPH within the context of New Zealand's technology export sector. TIN100

Earnings Calls & Primary Sources (via Quartr)

Your Quartr access gives you audio, transcripts, and slides for all key FPH events. Prioritize:

1. **H1 2026 Earnings Call (Nov 2025)** - Most recent results. Quartr
2. **H2 2025 / FY25 Earnings Call (May 2025)** - Full-year results with annual report. Quartr
3. **AGM 2025 (Aug 2025)** - Lewis Gradon's most candid forward-looking commentary. Quartr
4. **Investor Day 2022 (May 2022)** - The most comprehensive strategic deep-dive. Quartr

Suggested Learning Path: Start with the Outback Analyst deep dive. Listen to the 2024 Shared

Lunch episode. Work through the Investor Day 2022 slides on Quartr. Read the ERS/ESICM clinical

practice guideline to understand why nasal high flow is becoming standard-of-care. Follow that with the two most recent earnings calls. Finally, read the Sleep Docs piece on Philips' CPAP return for homecare competitive dynamics.

8. Corporate Culture

Founder Imprint and Engineering DNA

Fisher & Paykel Healthcare's cultural roots trace back not to a conventional medtech startup but to an appliance manufacturer's workshop. In the late 1960s, Dr Matt Spence noticed his ventilated patients suffering from dry, infected tracheas, and turned to Dave O'Hare, an engineer at Fisher & Paykel Industries. Together they built a prototype humidifier from a fruit preserving jar. When the healthcare division was demerged and separately listed in 2001, that collaborative, engineer-solves-a-clinical-problem ethos became the founding myth. The imprint persists with unusual clarity. CEO Lewis Gradon joined as an R&D engineer over 40 years ago, spent 15 years as SVP of Products & Technology, and six years running R&D before becoming CEO in 2016. This is not a company that recruited a turnaround specialist. Gradon's entire career - physics degree, bench engineering, product development, manufacturing, IP management - was spent inside the same organisation.

Decision-Making: Deliberate, Centralised, and Patient

FPH's decision-making style is best described as deliberate and engineering-led. The company has explicitly rejected an M&A-driven growth model. Management has stated: "We are not experts in merger and acquisition and don't want to be distracted from the size of the opportunity." Revenue has compounded at approximately 12% annually over the past decade - entirely organically. Glassdoor reviews corroborate this picture, though not always favourably. Recurring themes include "very slow decision process that jeopardizes momentum and motivation" and "very old school processes and tools." Career progression is described as taking a long time within a flat organisational structure. For investors, the question is whether deliberateness is a competitive moat (protecting against poor capital allocation) or a handicap (missing adjacencies). The 55-year track record of compounding suggests the former, but it demands monitoring.

Talent and Retention

FPH employs over 5,000 people globally, with approximately 1,000 in R&D - overwhelmingly based at Auckland. The company carries a 4.0/5.0 Glassdoor rating across 514 reviews, with 75% of employees recommending it. Culture and values scores (4.1) and work-life balance (4.1) are relative strengths; compensation (3.5) and career opportunities (3.5) are weaker. Employee turnover at approximately 15% exceeds the internal 10% target. Management deploys profit-sharing (\$15M in FY25, \$9M in H1 FY26), employee share plans (1.1M shares issued in H1 FY26), and on-site psychologists. The DiversityWorks NZ Inclusive Workplace

award (2022) reflects substantive commitment. However, below-target turnover and Glassdoor complaints about slow career progression suggest high-potential mid-career talent may find the culture constraining.

Innovation Culture: Internal, Clinical, and Evidence-Obsessed

FPH's innovation model is rare in medtech: almost entirely internal, clinically anchored, and evidence-driven. The patent portfolio is formidable: 6,758 patents globally across 1,047 patent families, with 5,749 active. The USPTO grant rate of 91.4% is exceptionally high. The clinical evidence strategy is the approach's crown jewel. Of 34 published studies supporting NHF therapy guidelines, 33 used Fisher & Paykel systems. This creates a self-reinforcing cycle: clinical evidence drives guideline adoption, which drives hospital purchasing, which funds further R&D. The company has made no material acquisitions in its history.

Integrity and Ethics

The record is not spotless but is manageable. In 2014, FPH dismissed a former Area Sales Manager who subsequently pleaded guilty to charges relating to NZD \$213,000 in unlawful payments connected to Middle East distributors. The company self-reported the issue and cooperated with New Zealand's Serious Fraud Office. Separately, FPH paid a USD \$50,000 settlement to the U.S. HHS Office of Inspector General. These are not trivial matters, but the company's response - self-reporting, dismissal, cooperation - suggests a culture that treats compliance failures seriously.

The COVID Stress Test

FPH was designated an essential service during New Zealand's Level 4 lockdown in March 2020. Hospital hardware revenue grew over 400% in FY2021. The harder cultural test came after the surge. FY2023 saw a revenue hangover as hospitals digested stockpiles. The company held its R&D spending, continued infrastructure investment, and avoided significant layoffs - demonstrating counter-cyclical discipline that distinguishes genuine long-term cultures.

Red Flags

Three areas warrant scrutiny. First, CEO succession: Gradon has been with the company 42 years. Second, geographic culture risk: replicating Auckland-centric culture across Mexico, China, and other markets is challenging. Third, the slow-decision-making culture may become costly as competitive intensity increases in digital health and connected devices.

9. Earnings Call Deep Dive

Narrative Evolution

Over the past four reporting periods, management's narrative has undergone a decisive shift from recovery to confident offence. At the H2 FY24 result (May 2024), FPH was still absorbing the

post-COVID demand cliff: NPAT had collapsed to \$133M, and the story was about bottoming out. By H1 FY25 (November 2024), the narrative pivoted sharply - revenue of \$951M was up 18%, and language shifted to "record" territory. CEO Gradon began emphasising clinical practice change as the structural growth engine.

At FY25 (May 2025), management delivered at the top end: \$2,021M revenue, \$377M NPAT. New product launches - Nova Nasal, Airvo 3, the 950 system - were highlighted. The narrative introduced a new emphasis on anesthesia as a growth vertical. By H1 FY26 (November 2025), the story reached a new peak: first-ever half above \$1 billion.

What has been quietly dropped: any reference to the COVID demand hangover. The Airvo 2 recall has faded from the narrative. The theme amplified across every call is the long runway - clinical practice change, the \$25B+ TAM, and the recurring consumables model.

Guidance and Delivery

Management has consistently delivered at or above guidance:

- **FY25 guidance:** \$1.9-2.0B revenue, \$320-370M NPAT. **Delivered:** \$2,021M, \$377M - above top end.

- **FY26 guidance:** Initially \$2.15-2.25B / \$390-440M, upgraded to \$2.17-2.27B / \$410-460M after strong H1.

- **Gross margin trajectory:** 60.5% in FY24 -> 61.9% in H1 FY25 -> 62.9% FY25 -> 63% H1 FY26.

Remarkably steady ~100bps improvement per half.

Analyst Concerns

Tariffs and trade policy dominate from H2 FY25 onward. Management responds with precision (32bps H1, 75bps full year) but does not speculate on policy changes.

OSA mask competitive dynamics are perennial. Management acknowledged growth "slowed in the second half due to competitor launches," guiding to ~9% growth - a meaningful step-down from 14-16%.

Hospital hardware pull-forward risk surfaced at H1 FY26, with management explicitly flagging that hardware sales "may represent a pull-forward of demand, not expected to repeat in H2."

Seasonal hospitalisation sensitivity is the question management redirects to most frequently, noting it can swing results by "up to 5% of the business."

GLP-1 weight loss drug impact on OSA was raised at H2 FY25. Management dismissed it: no observed impact on mask usage.

Key Reveals

1. **Anesthesia as a breakout vertical.** At H1 FY26, anesthesia grew ~40% and now accounts for over 10% of new applications consumables - a material acceleration of an underdiscussed business line.

2. **The RENOVATE study's strategic significance.** Confirmed nasal high flow as non-inferior to NIV - potentially the single most important clinical catalyst in the company's pipeline.

3. **Hardware pull-forward admission.** Management explicitly stated H1 FY26 hardware sales "may represent a pull-forward" - unusual candour.

4. **OSA mask growth normalisation.** First explicit acknowledgement that competitive intensity in masks has increased.

5. **Tariff quantification precision.** Management quantified at exactly 32bps for H1 and 75bps full year, demonstrating the headwind is real but dwarfed by operational improvements.

Q&A Dynamics

The most telling dynamic is what analysts have stopped asking about. No one probes COVID comparisons. No one questions whether gross margin recovery is sustainable. The debate has shifted from "can they recover?" to "how fast and for how long?" - exactly the transition management has engineered.

10. Strategic Documents Analysis

Strategic Priorities

Fisher & Paykel Healthcare's strategy rests on five interlocking pillars consistent since the 2022 Investor Day: (1) changing clinical practice through evidence-based NHF adoption; (2) expanding into adjacent hospital applications, particularly anesthesia; (3) growing homecare market share in OSA masks; (4) investing in manufacturing scale and geographic diversification; and (5) sustaining a high-reinvestment R&D model at ~11% of revenue.

The hierarchy has shifted subtly. At the 2022 Investor Day, the dominant narrative was converting the COVID-era installed base into recurring consumables. By FY25 and H1 FY26, the messaging has matured: the installed base conversion is delivering (74% new applications), and management has layered on anesthesia and the Airvo 3 platform as next growth vectors. The strategy has not changed direction; it has shifted emphasis from aspiration to execution proof points.

Capital Allocation Framework

Management describes capital allocation as overwhelmingly organic. The company has never made a material acquisition - striking for a \$21B market cap healthcare equipment company. Capital priorities: R&D reinvestment (\$227M, 11% of revenue), capacity expansion (~\$150M capex plus \$700M land-and-buildings program over five years), dividends (42.5 cps, ~65% payout), and no buybacks.

The numbers support the rhetoric. R&D stayed at 11% through the cycle, even when earnings were compressed. The \$275M Karaka land purchase for a 30-40 year campus signals genuine long-horizon thinking. The absence of buybacks at elevated multiples is rational.

Long-Term Targets

The central aspiration is to double constant currency revenue every five to six years (12-15% CAGR). Against FY25 revenue of \$2.02B, the implied target is ~\$4B by FY30-31. The gross margin target of 65% has been consistent - from 59.3% trough in FY23, management has rebuilt to 63% in H1 FY26, pointing to achievement by FY28.

These targets are credible but not conservative. There is no explicit operating margin or ROIC target in public communications, which limits accountability on capital efficiency.

Narrative vs Reality

The strongest alignment is the hospital consumables story - management talked about COVID-installed-base conversion since 2022, and the data confirms it. The area of tension is gross margin: management treats 65% as almost inevitable, but tariff uncertainty introduces variables beyond management's control. Homecare mask growth (slowing to 6% in recent periods) also warrants scrutiny against the consistently optimistic narrative.

Addressable Market Framing

Management frames TAM at \$25B+, constructed from credible building blocks. At the 2022 Investor Day, Gradon claimed the anesthesia patient population is comparable to general respiratory patients - effectively doubling the hospital TAM. The TAM framing is more specific and evidence-grounded than most medtech peers, but management does not routinely disaggregate serviceable from total addressable market.

Risk Acknowledgement

Strategic communications have historically been light on risk discussion. Notably absent: substantive discussion of GLP-1 drugs and OSA impact, competitive intensification from ResMed, single-country R&D concentration risk, or USMCA 2026 review implications for Mexico manufacturing. The company's risk disclosures cover standard items but do not reflect the strategic risks sophisticated investors are actually pricing.

Evolution of Messaging

The arc from 2022 to present reveals a company that set its strategic framework during COVID-inflated revenue and has demonstrated it holds in normalised demand. By FY25, the messaging evolved from "here is the opportunity" to "here is the evidence." The most significant shift is around tariffs - the 2022 Investor Day was delivered in stable trade policy; current communications must address a fundamentally changed tariff environment, making the overall strategic communication more balanced and arguably more credible.